

Discharge Summary

Patient:	Siegfried Wagner
Date of Birth:	01/08/1958
Admission Date:	10/28/2024
Discharge Date:	11/06/2024
Attending Physician:	Prof. Andreas Engert, MD

Dear Colleagues,

Mr. Siegfried Wagner, DOB 01/08/1958, was admitted on 10/28/2024 to initiate R-CHOP immunochemotherapy for newly diagnosed diffuse large B-cell lymphoma.

Mr. Wagner was referred 6 weeks ago with a 3-month history of rapidly progressive bilateral cervical lymphadenopathy, drenching night sweats, and 8 kg weight loss. Excision biopsy confirmed GCB-type DLBCL, MYC-negative, Ki-67 80%. PET-CT staging: bulky mediastinal and para-aortic disease with liver involvement.

Primary Diagnosis: Diffuse large B-cell lymphoma (C83.3) — Tumor staging: T3 N2 M1

Laboratory values:

HbA1c	6.0%
LDH	412 U/L
Creatinine	0.9 mg/dL

Past Medical History: Well-controlled type 2 diabetes on metformin 850 mg twice daily (HbA1c 6.0%). Mild hypertension on amlodipine 5 mg. No prior malignancy. Non-smoker. Retired civil engineer, lives with his wife of 32 years.

Cardiac Assessment Prior to Anthracycline: Echocardiography day 1: EF 62%, no significant valvular pathology, normal LV dimensions. ECG: normal sinus rhythm. Cleared for doxorubicin-containing therapy.

Hospital Course: Cycle 1 of R-CHOP administered over 3 days. Pre-medication: ondansetron 8 mg IV, dexamethasone 8 mg IV, allopurinol 300 mg (TLS prophylaxis). Grade 1 nausea day 2, managed satisfactorily with oral ondansetron.

Appetite returned by day 3. No febrile episodes. BP and glucose well controlled throughout. Metformin held on admission, reinitiated at discharge.

Patient and Carer Education: Hematology specialist nurse provided comprehensive education session on day 5 covering the R-CHOP treatment schedule (6 cycles planned), expected side effect profile including haematological toxicity, infection risk, peripheral neuropathy, hair loss, and infusion reactions; G-CSF administration; febrile neutropenia recognition (temperature above 38°C requires immediate emergency assessment); and precautions during neutropaenic nadirs. Mr. Wagner and his wife attended and asked detailed questions. A patient information booklet with key telephone numbers was provided.

G-CSF (filgrastim 300 mcg subcutaneously) was prescribed for 5 days starting 48 hours post-chemotherapy.

Medication at Discharge: Metformin 850 mg twice daily, amlodipine 5 mg, allopurinol 300 mg (4 weeks), filgrastim 5-day course, ondansetron as needed, pantoprazole 40 mg.

Follow-up: Hematology in 3 weeks for pre-cycle 2 assessment. CBC in 10 days. Interim PET-CT after cycle 2. Seek urgent care if temperature exceeds 38°C.

Supportive Care During Chemotherapy Administration:

The oncology nursing team monitored Mr. Wagner continuously during chemotherapy. IV access was maintained via a peripheral cannula throughout the admission. A discussion regarding implantable port insertion for future cycles was held — the patient preferred to defer this decision until after the second cycle. Antiemetic prophylaxis was effective and no IV rescue antiemetics were required. Vital signs were recorded hourly during rituximab infusion and every 4 hours on chemotherapy days. No infusion-related reactions occurred during rituximab administration.

Steroid-Related Glycaemia and Future Cycle Planning:

Mr. Wagner's blood glucose was monitored four times daily on days when prednisolone 100 mg was being administered. Steroid-induced hyperglycaemia was mild — peak glucose 218 mg/dL on day 2 of chemotherapy — and did not require insulin intervention. Metformin was held throughout the admission and

reinitiated at discharge. The diabetes management plan for subsequent cycles specifies: intensified glucose monitoring on prednisolone days; metformin to be held 48 hours before any contrast-enhanced CT; a glucose threshold of 250 mg/dL above which the patient should contact the diabetes team for sliding scale insulin advice.

Future Treatment Plan and Response Assessment:

The total planned treatment is 6 cycles of R-CHOP at 21-day intervals. An interim PET-CT will be performed following cycle 2 to assess early metabolic response. A complete metabolic response at interim PET is associated with excellent outcomes in DLBCL. End-of-treatment PET-CT will be performed 6–8 weeks after cycle 6. CNS prophylaxis with intrathecal methotrexate is not indicated based on the CNS-IPI score of 2. The patient and his wife were fully informed of the treatment schedule, the role of interim imaging, and the expected outcomes.

Prof. Andreas Engert, MD

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